

# Integrating child mental health responses into recovery, development, and peacebuilding in fragile and conflict affected settings

**Zulfiqar A Bhutta and colleagues** call for renewed global commitments and funding mechanisms to build on promising initiatives to protect children affected by armed conflict

**T**he world is a challenging place for children today, with a staggering 473 million living in fragile and conflict affected settings worldwide.<sup>1</sup> Witnessing or enduring violence during their most formative years of development makes children especially vulnerable to lasting mental health and psychosocial disorders. Yet less than 1% of development assistance is allocated to mental health services, and many children affected by conflict do not have access to evidence based mental health and psychosocial support (MHPSS) interventions.<sup>2</sup>

The BRANCH Consortium, an academic research enterprise aimed at improving evidence for effective action on women's and children's health and nutrition in

conflict settings, has comprehensively documented the health effects of conflict on women and children and identified critical gaps in intervention.<sup>3</sup> These findings confirm what practitioners have long observed: we know what works, but we fail to deliver at scale. In addition, we are failing to connect humanitarian, development, and peace efforts, meaning responses to acute emergencies continue to be delinked from the recovery, development, and the peacebuilding agenda. This is related to insufficient integration of evidence based mental health and psychosocial services for affected children within health, education, and social protection services and, importantly, to insufficient investment in systems to care for war affected children, youth, and families over the longer term.

Other articles in this BMJ Collection investigate the pathways through which children's mental health is affected by early adversity in fragile and conflict affected settings and the evidence based interventions and initiatives needed to treat these outcomes.<sup>4 5</sup> Here we consider how to integrate mental health support into the humanitarian, development, and peace nexus to close gaps in provision.

## Including mental health in humanitarian, development, and peace nexus

The humanitarian, development, and peace nexus (HDPN) refers to a three pronged approach that is intended to strengthen collaboration between humanitarian, development, and peace organisations and activities as part of the United Nations' effort to establish a new way of working.<sup>6</sup> The World Health Organization (WHO) defines the HDPN as "any health related activity where at least two of the three groups of actors (humanitarian, development, and peace) work together with the aim of providing immediate lifesaving and life supporting assistance; strengthening or rebuilding national systems, institutions, and capacities; strengthening emergency

management capacities; or addressing the drivers of emergencies."<sup>6</sup> Although these sectors have historically worked in siloes, the rise in fragile and conflict affected settings shows the need to link immediate humanitarian response with long term rehabilitation and crisis prevention, and support for children and families in affected populations must be prioritised.

MHPSS programmes for children affected by conflict require shared leadership and financing to implement multiyear planning and programming for health protection and sustainable development. Developing a well articulated and evidence based continuum of care is critical to ensuring that different mental health needs are supported long term to foster resilience and recovery. This approach has also been recognised by the United Nations Children's Fund (Unicef) as relevant to scaling its global multisectoral operational framework for mental health support of children, adolescents, and care givers.<sup>7</sup> The core principles of the Inter-Agency Standing Committee's guidelines for MHPSS in emergencies (human rights and equity, participation, do no harm, build on local capacities and resources, integrated support systems, and multilayer supports) highlighted in this framework outline the necessity for equity informed implementation that considers the age, developmental stage, gender, and vulnerability of conflict affected children.<sup>8</sup> While current guidelines are strong, an organised and intentional implementation science initiative is needed to scale and sustain quality of evidence based MHPSS services for children and families modelled on the Global Fund's approach to sustainable health and community systems and the US National Child Traumatic Stress Network.<sup>9 10</sup>

WHO has made important efforts to implement an HDPN approach in six countries in the Middle East and Africa.<sup>8 11</sup> In addition, the following case studies show how countries have

## KEY MESSAGES

- The humanitarian, development, and peace nexus (HDPN) offers new opportunities to reimagine sustainable mental health and psychosocial support systems of care for children in conflict settings
- The greatest challenges continue to be chronic underinvestment, weak coordination across sectors, and lack of clear accountability measures and consequences for perpetrators of violence
- A global funding mechanism is needed to ensure adequate financing for mental health support and integration of such interventions within HDPN platforms, as well as health and education systems
- A global convening of leaders is urgently needed to gain consensus on actionable steps to ensure implementation of existing protocols for prevention of harm to children in conflict settings and ensuring investments to address the mental health and developmental consequences

explicitly attempted to harmonise funding and collaboration across the nexus by extending immediate MHPSS response among youth into primary care and rehabilitation services, including in active conflict, fragile displacement, and post-conflict contexts.

### Case studies of integration

#### Ukraine

Russia's 2022 invasion of Ukraine led to millions of children experiencing trauma and displacement. This has galvanised concerted local responses within Ukraine, such as First Lady Olena Zelenska's launch of the national programme of mental health and psychosocial support to integrate mental health into schools and primary health-care, train teachers and psychologists, and deploy mobile crisis teams. Her partnerships with Unicef and WHO have created "Spilno spots": child friendly spaces and trauma rehabilitation programmes that have reached over 500 000 children.<sup>12</sup> Convened under her leadership, the 2023 Mental Health Summit brought together global stakeholders to coordinate long term strategies and advocate for policy reform for sustainable MHPSS funding and infrastructure investments, including more trauma informed training for Ukraine's existing MHPSS workforce.<sup>13</sup>

Interviews with Spilno beneficiaries reported that these efforts have improved children's psychological wellbeing by reducing anxiety, aggression, and sleeping difficulties, while also improving socialisation and self-confidence.<sup>12</sup> However, this remains the only evaluation conducted to assess effectiveness, and a 49% (\$403m) funding gap represents a major challenge to sustaining these activities. Although some facilities have now been transformed into government owned multipurpose and resilience centres where families can continue receiving some of Spilno's services, the closure of several Spilno spots has been a major limitation to ensuring consistent mental health support.<sup>12</sup> Nonetheless, such initiatives lay the groundwork for strengthening sustainable and responsive systems in active conflict regions, with local policy makers leading efforts to bridge humanitarian response, peacebuilding, and development.<sup>14</sup>

#### India

The United Nations High Commissioner for Refugees has collaborated closely with the government of India to support over 46 000 refugees and asylum seekers from Afghan-

istan, Myanmar, and Sri Lanka to resettle within the country.<sup>15</sup> Investments have been focused on integrating MHPSS into community engagement activities, particularly for youth, women, and people with disabilities. Local community health workers have been trained to provide critical psychosocial support for displaced populations as well as enabling them to access local health facilities remotely through a 24/7 helpline.<sup>15</sup> Out of the 272 community groups supported, almost half were specifically developed for child protection committees, youth clubs, and adolescent groups.<sup>15</sup>

Although these efforts initially began through humanitarian response mechanisms, they show that MHPSS pathways can be developed to empower communities to be active participants in their protection, recovery, and rehabilitation. Similar to the Ukraine context, however, a 60% (\$54m) funding gap remains a major barrier to implementing these activities and evaluating their effectiveness. No evaluations have been conducted to assess the mental health effects of these interventions among displaced children in India. Long term assessment needs to be prioritised to ensure that the limited resources are used to implement the most effective MHPSS interventions.

#### Sierra Leone

Anticipatory action mechanisms have traditionally been used in response to natural and climate related hazards whereby early warning systems are in place to monitor and predict the onset of disasters.<sup>16</sup> When a pre-agreed set of conditions are met, pre-pooled financing is released before a disaster to ensure communities are prepared and remain resilient to any consequences.<sup>16</sup> The Start Network has made extensive efforts to translate this initiative for fragile and conflict affected settings. Its "no/low regrets" approach follows a flexible funding structure that relies on context specific risk analyses and collective expert decision making to allow for the proactive implementation of conflict prevention, resilience, and peacebuilding interventions.<sup>16</sup> For instance, in anticipation of violence related to the 2023 election in Sierra Leone, 700 youth were trained in conflict management, and 1750 youth became peace ambassadors, leading election related dialogues to reduce community tensions, counter misinformation, and spread messages of non-violence in their communities.<sup>16</sup>

Although evaluation of the Start Network initiative is not publicly available, similar

peacebuilding initiatives in Sierra Leone that used youth ambassadors to promote changes in community behaviours and attitudes report that participants gained a renewed sense of purpose while also improving self-esteem, confidence, and communication skills.<sup>17</sup> By contrast, post-conflict reconciliation initiatives in Sierra Leone that have implemented community level forums to discuss conflict related experiences and atrocities have identified opposite effects on psychological wellbeing. Although dialogue led to significantly greater forgiveness of perpetrators of violence and expanded social networks, participants sharing their experiences had higher incidences of depression, anxiety, and post-traumatic stress disorder.<sup>18</sup> This shows the need to ensure recovery, reconciliation, and resilience interventions inherently prevent re-traumatisation, provide clear pathways to specialised services when needed, and ensure justice is promised to victims of atrocities.

### Challenges and opportunities

These case studies show the challenges of integrating mental health support into humanitarian, development, and peacekeeping activities—lack of funding, evaluation, and accountability. Progress is primarily inhibited by the lack of wide recognition and commitment among donors, leaders, state institutions, and civil society organisations to devise new mechanisms of knowledge and resource sharing at subnational, national, and international levels.<sup>6</sup> Further, the HDPN faces massive reductions in humanitarian and development assistance from most major donors related to cascading funding cuts initiated by the US government, growing donor fatigue, working in dangerous and unpredictable environments, and continuing political uncertainty about sources of funding and support for their work.<sup>19</sup> This includes security and safety issues, as humanitarian and peacekeeping workers are often targeted and attacked while trying to ensure the protection of affected populations.<sup>20</sup>

We must do everything possible to prevent violence and atrocities against children in all fragile and conflict affected settings, irrespective of the polarised political environment. Established common humanitarian principles (humanity, impartiality, neutrality, independence) are increasingly challenging to uphold given partisan positions.<sup>21</sup> The long tortuous road to a tenuous ceasefire in Gaza after almost three years of incessant

attacks on civilians and a stranglehold over humanitarian assistance, including food and water, is a tragedy.<sup>22</sup> The political inertia of the current Sudanese civil war, characterised as the worst humanitarian crisis in the world with 15.6 million children requiring assistance, is another case in point.<sup>23</sup>

Greater south-south collaboration could enable MHPSS at scale and build more effective multisectoral integration and coordination. For instance, the West African Network for Peacebuilding (members from Benin, Burkina Faso, Cape Verde, Cote d'Ivoire, Gambia, Ghana, Guinea, Guinea-Bissau, Liberia, Mali, Niger, Nigeria, Senegal, Sierra Leone, and Togo) was established to prevent conflict and promote peace after the region's tumultuous history of violence in the 1990s. The network's collaborative focus has resulted in shared financing for region-wide programmes, including MHPSS related capacity building initiatives involving peer mediation, mentorship, and education among youth.<sup>24</sup>

We also need to identify sufficient funding for focused implementation science initiatives to help implement and sustain quality mental health services for children and families in fragile and conflict affected settings. Robust country plans for strengthening mental health support need to be developed in each affected setting, akin to the plans used for health systems strengthening and drawing from national coordination mechanisms, especially in settings with limited mental health professionals.<sup>25</sup> These mechanisms intentionally connect research institutions from low, middle, and high income countries with frontline provider organisations to train and support their capacity to deliver evidence based MHPSS services—as well as developing, testing, and adapting strategies for integration within other delivery platforms such as schools, livelihoods programmes, social protection, nutrition, and primary care.<sup>10</sup> A global initiative of this nature can be launched by enabling financing and incentivising partnerships and coordinating mechanisms to develop regional groups such as the West African Network for Peacebuilding, which can learn and deliver evidence based interventions. This process needs to be country led with local stakeholders, including local governments and civic society organisations engaged in activities.

Such efforts will also need global consensus to move forward and generate

the political will and resources to explicitly address the needs of children. The 1990 world summit for children set the scene for the global child survival strategy and the millennium development goals. Likewise, a seminal UN report in 1996 on children affected by conflict drew attention to grave violations facing children in war and a subsequent *Lancet* series called for concerted efforts to support multisectoral planning for child health.<sup>26</sup> Although the first global ministerial conference on ending violence against children in 2024 had support from over 100 governments, only 15 countries (Burundi, Bhutan, Botswana, Cape Verde, Equatorial Guinea, Kenya, Liberia, Mozambique, Solomon Islands, Trinidad and Tobago, Turkey, South Sudan, the US, United Arab Emirates, and Zambia) made pledges related to MHPSS implementation, which remains fragmented.<sup>27</sup> We need renewed recognition and reaffirmation of existing international humanitarian laws, regulations, and principles that were put in place to protect children.<sup>28</sup> These are meaningless in the absence of a mechanism to implement them and to bring perpetrators of war crimes to justice.

Notwithstanding the widespread mistrust of summits and photo opportunities, there is an urgent need to bring together key global leaders, UN agencies, and philanthropic funders to arrive at a consensus and establish a focused effort to scale evidence based MHPSS systems for children and families in fragile and conflict affected settings. Regional powers, civic society groups, and development banks should also participate in any meeting. Key themes for discussion include addressing the physical and mental health impacts of childhood exposure to armed conflict, developing a targeted initiative to scale effective and evidence based interventions to strengthen systems of MHPSS for children and their families during and after conflict, and expanding existing protections for children by appropriately investing in, and evaluating initiatives to, integrate MHPSS with humanitarian, development, and peacemaking efforts and reinforcing accountability frameworks within international commitments. Given the long term and intergenerational costs of developmental consequences for children living in fragile and conflict affected countries, building a consensus on action is a necessary investment.

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